Thyroid Examination — OSCE Checklist

Deakin MD2 · ELC5 · Time: 8 min (+2 min reading)

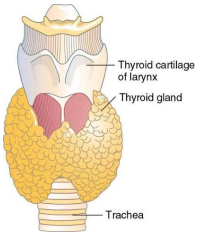
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Hands → Vitals/Arms → Eyes/Face → Neck → Chest → Legs**. *Look for* **hyper- AND hypothyroid** *signs at every step and say what you do / don't see.*

1. Introduction

☐	**W**ash hands
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**E**xpose neck to clavicles; **P**osition seated on a chair (room to stand behind)
☐	Ask about pain / neck tenderness; have a glass of water ready

2. Examination

	Step	What to do / say	Abnormal
General inspection			
☐	Body habitus	Signs of weight change	Loss (hyper); gain not necessarily more common (hypo)
☐	Clothing	Dressed for the weather? Heat / cold intolerance	Layers = hypo; sweaty, few clothes = hyper
☐	Well / unwell	Anxiety, agitation, restlessness	Hyper
☐	Obvious neck mass	Goitre or thyroglossal cyst	May be seen before regional exam
Hands			
☐	Nails	Clubbing (thyroid acropachy), onycholysis	Hyper (Graves)
☐	Skin	Palmar erythema, warm, sweaty	Hyper
☐	Skin	Peripheral cyanosis, palmar pallor, yellowing (hypercarotenaemia)	Hypo
☐	Fine tremor	Arms outstretched, sheet of paper on the backs of hands	Hyper (sympathetic overactivity)
Vital signs and arms			
☐	Radial pulse (manual)	Rate, rhythm, volume	Tachycardia / AF (hyper — confirm on ECG); slow, small volume (hypo)
☐	Vital signs	BP, RR, SpO_2, temp	Wide pulse pressure, fever (hyper) †
☐	Arm reflexes	Biceps, brachioradialis	Brisk (hyper); slow relaxation (hypo) †
☐	Proximal myopathy	Arms overhead / resisted "chicken wings" (can do with legs)	Hypo or hyper
Eyes and face			
☐	Face / skin	Pallor, yellowing, thickened skin, alopecia, vitiligo, xanthelasma	Hypo; vitiligo = autoimmune
☐	Periorbital / eyebrows	Periorbital oedema; loss of outer third of eyebrows	Hypo
☐	Tongue / speech	Macroglossia, hoarse or slow speech	Hypo
☐	Exophthalmos	Look from front, side and above/behind	Graves' disease only
☐	Lid retraction / stare	Sclera visible above iris	Hyper
☐	Lid lag	Follow finger from high to low	Hyper (sympathetic, Müller's muscle)
☐	Conjunctiva / cornea	Chemosis, conjunctivitis, corneal ulceration	Graves (can't close eyes)
☐	Eye movements + ptosis	H test, ask about diplopia	Ophthalmoplegia (Graves); ptosis — consider myasthenia
Neck — inspect (front and sides)			
☐	Scars, redness, veins	Thyroidectomy scar; redness (thyroiditis); prominent veins	Retrosternal goitre
☐	Swelling	Note any goitre or mass	Diffuse vs nodular
☐	Swallow	"Take a sip of water and hold it… now swallow"	Goitre and thyroglossal cyst move up
☐	Tongue protrusion	"Stick out your tongue"	Thyroglossal cyst moves up; goitre does not
Neck — palpate (from behind)			
☐	Thyroid	Warn patient; both lobes + isthmus: size, shape, consistency, symmetry, tenderness, nodules	Diffuse (Graves), multinodular, solitary nodule
☐	Swallow	Palpate while patient swallows water	Tethering = possible malignancy
☐	Lymph nodes	Cervical and supraclavicular chains	Malignancy
☐	Trachea*	Position †	Deviation from large goitre



Thyroid gland anatomy

Neck — percuss and auscultate			
☐	Percuss	Over upper sternum	Dullness = retrosternal goitre
☐	Pemberton's sign	Arms up beside ears for 1 min: facial plethora, cyanosis, neck vein distension, stridor	Thoracic inlet obstruction
☐	Auscultate	Over each lobe with diaphragm for a bruit	Bruit = hyper (Graves)
Chest			
☐	Gynaecomastia	If not already seen	Hyperthyroidism, hypogonadism
☐	Heart / lungs	Auscultate for pericardial and pleural effusions	Hypo
Legs			
☐	Pretibial myxoedema	Shins: raised, waxy, orange-peel plaques	Graves
☐	Oedema	Press over shin	Non-pitting = hypo
☐	Proximal myopathy	Stand from chair, arms crossed	Hypo or hyper
☐	Reflexes	Knee, ankle	Brisk (hyper); slow relaxing (hypo)

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion: "I would check TFTs (TSH, free T4 ± T3), thyroid antibodies, and arrange a thyroid ultrasound if a goitre or nodule is found." †
☐	Present: state thyroid status first, then the neck: "Mrs X appears clinically euthyroid. There is no goitre, the neck is non-tender with no lymphadenopathy, and no eye signs of Graves' disease."

4. Quick reference

Region	Hyperthyroid	Hypothyroid
General	Weight loss, anxious, agitated, heat intolerance	Weight gain, slow, cold intolerance, dressed warmly
Hands	Sweaty, warm, palmar erythema, onycholysis, acropachy, fine tremor	Cool, dry, pale, yellow (carotenaemia), peripheral cyanosis
Pulse / reflexes	Tachycardia, AF; brisk reflexes	Bradycardia, small volume; slow-relaxing reflexes
Eyes / face	Lid retraction, lid lag; Graves: exophthalmos, chemosis, ophthalmoplegia	Periorbital oedema, loss of outer ⅓ eyebrows, coarse thick skin, alopecia
Neck	Goitre ± bruit (Graves)	Goitre (Hashimoto's) or none
Legs	Pretibial myxoedema (Graves), proximal myopathy	Non-pitting oedema, proximal myopathy

- Goitre: 80% euthyroid, 10% hypo, 10% hyper. Isthmus lies over tracheal rings 2–4.
- Only **Graves' disease** causes exophthalmos and pretibial myxoedema; lid lag and stare occur in any hyperthyroidism.
- Pitfalls: palpating from the front only, forgetting the swallow and tongue tests, not commenting on thyroid status, skipping the legs.

Menstrual History — OSCE Checklist

Tick each item as you ask it. **Shaded rows (*)** *are asked if relevant.* † *= added from standard teaching, not in the Deakin guideline. Use sensitive, non-judgemental language and establish the patient's goals and concerns.*

1. Opening

☐	Introduce self, confirm patient ID, explain purpose; consent
☐	Signpost: "Some of these questions are personal — you don't have to answer anything you're not comfortable with."
☐	Open question: "Do you have periods or a menstrual cycle?"

2. The cycle

	Ask about	Example question	Notes
☐	Menarche	"How old were you when your periods started?"	
☐	Menopause*	"Have your periods stopped?"	Menopause = 12 months since the final period; before this = perimenopause
☐	LMP / LNMP	"When did your last normal period start?"	Any chance of pregnancy?
☐	Regularity	"Do they come at about the same time each month?"	Regular vs irregular
☐	Cycle length	"How many days from the start of one period to the start of the next?"	Normal ~21–35 days
☐	Duration	"How many days do you bleed for?"	Normal ~4–7 days (30–80 mL)
☐	Tracking	"Do you track your cycle, e.g. with an app?"	Helps with oligo-/amenorrhoea

3. Problems with periods

	Ask about	Example question	Notes
Heavy menstrual bleeding (HMB) — "Is it too heavy?"			
☐	Clots, flooding, leakage	"Do you pass clots or leak through?"	Suggests HMB
☐	Products	Pads, tampons, cups, period underwear; using two at once?	Dual products = heavy flow
☐	Frequency of change	"How often do you change, and are they soaked?"	Change <2-hourly = significant
☐	Impact	Physical, emotional, social, work/school	Defines HMB
☐	Anaemia symptoms*	Fatigue, breathlessness, dizziness †	Iron deficiency
Dysmenorrhoea — "Is it too painful?"			
☐	Timing and site	When does pain start (may precede bleeding by days)? Where? How long?	
☐	Impact	Missing school or work? What helps?	
☐	Associated symptoms	Nausea, vomiting, diarrhoea, constipation	Consider endometriosis
Cycle too frequent or infrequent			
☐	Oligo- / amenorrhoea	Cycle length; periods stopped? Pregnancy?	Consider PCOS, thyroid, weight change †
Abnormal bleeding — red flags			
☐	Intermenstrual (IMB)	Bleeding between periods?	Cervical / endometrial pathology
☐	Post-coital (PCB)	Bleeding after sex?	Cervical pathology
☐	Post-menopausal (PMB)	**Any** bleeding or spotting after menopause?	Red flag — exclude endometrial cancer urgently

4. Management of periods

☐	Hormonal	COCP, POP, IUD, implant, injection — not always used for contraception	All can alter the cycle
☐	Non-hormonal	NSAIDs, tranexamic acid	Pain and flow control

5. Link to related history and close

☐	Gynaecological, obstetric, sexual health (± gender identity) and surgical history as relevant
☐	Thyroid symptoms* (heat/cold intolerance, weight change) — thyroid disease can change menstrual bleeding
☐	Ideas, concerns, expectations: what does the patient want? (some wish to have, others not to have periods)
☐	Summarise back, check accuracy, invite questions, thank patient

6. Quick reference

Type	Causes of HMB
Structural	Fibroids (leiomyomata), adenomyosis, endometrial polyps, endometrial hyperplasia, endometrial cancer
Non-structural	Ovulatory disorders (PCOS, perimenopause, puberty), clotting disorders, iatrogenic (IUD), hypothyroidism
Mnemonic †	PALM-COEIN

Term	Meaning
HMB	Excess loss interfering with quality of life
IMB	Bleeding between periods
PCB	Bleeding after intercourse
PMB	Any bleeding ≥12 months after the final period
Cycle length	Day 1 of one period to day 1 of the next

Diabetes Examination — OSCE Checklist

Deakin MD2 · ELC3 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Vitals → Hands → Face → Eyes → Neck/Chest → Abdomen → Lower limbs**. *Look for* **microvascular** *(eyes, kidneys, nerves) and* **macrovascular** *(heart, carotids, legs) complications.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain and gain consent; ask about pain
☐	Position and expose as for CV exam, plus abdomen and lower limbs (keep the patient covered and comfortable)

2. Examination

	Step	What to do / say	Significance
General inspection			
☐	Age, body habitus	Obesity, central adiposity; cachexia	T2DM, metabolic syndrome
☐	Acute distress	Hyperventilating, unwell (Kussmaul breathing); confused, anxious, sweaty	DKA; hypoglycaemia
☐	Gait	Observe as the patient enters	Neuropathy, PVD, foot problems
Vital signs and measurements			
☐	BP lying and standing	Postural drop	Autonomic neuropathy, dehydration
☐	HR, RR, SpO_2, temp	Full set	Sepsis, DKA
☐	BGL ± ketones	Ketones if BGL elevated	Ketones >0.6 mmol/L = red flag
☐	Weight, height, BMI, waist	If appropriate and able	Cardiometabolic risk
Hands and arms			
☐	Fingertips	Finger-prick marks; sites rotated?	Self-monitoring
☐	Dexterity	OA, limited joint mobility	Ability to self-inject / test
☐	Upper arm	Continuous glucose monitor (CGM)?	
Face			
☐	Endocrine facies	"Moon face"	Cushing's (secondary diabetes)
☐	Mouth	Infection: bacterial or Candida (white plaques)	Poor glycaemic control
Eyes			
☐	Visual acuity	Snellen chart, glasses on; each eye	Retinopathy, maculopathy, cataract
☐	Eye movements*	If clinically indicated	CN III / VI mononeuropathy
☐	Fundoscopy	Red reflex, disc, vessels, macula (note undilated-pupil limits)	Retinopathy, cataract
Neck and chest			
☐	Carotids	Auscultate for bruits	Carotid atherosclerosis
☐	Acanthosis nigricans	Brown velvety skin: back of neck, axillae, folds	Insulin resistance
☐	Precordium	Auscultate heart; ± lung bases if cardiac concern	Heart failure, IHD



Acanthosis nigricans on the neck — Axillary acanthosis nigricans

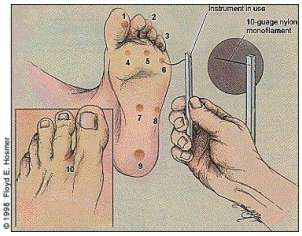
Acanthosis nigricans — neck and axilla

Abdomen			
☐	Injection sites	Clean? Rotated? Lipohypertrophy or lipoatrophy	Erratic insulin absorption
Lower limbs — inspect (incl. backs of legs, between toes, soles)			
☐	Gait and footwear	Gait; shoe type and fit	Pressure areas
☐	Nails and foot care	Nail condition; impingement on adjacent toes	
☐	Interdigital	Maceration, tinea pedis	Infection portal
☐	Skin	Dry fissured skin, atrophy, callus, necrobiosis lipoidica	Neuropathy, high pressure
☐	Pressure areas, ulcers	Heels, metatarsal heads, toes: ulcer, crack, erythema; depth?	Ulcer to tendon/bone = red flag
☐	Trophic changes	Hair loss, shiny skin	PVD
☐	Bone architecture	High arch, clawed toes, bunions, muscle wasting, Charcot joint	Motor / sensory neuropathy



Necrobiosis lipoidica

Lower limbs — vascular			
☐	Pulses	Distal → proximal: DP, PT, popliteal, femoral	PVD
☐	Perfusion	Capillary refill in toes; temperature of limbs	Critical limb ischaemia = red flag
Lower limbs — neurological			
☐	Monofilament (10 g)	Eyes closed; several plantar sites each foot (hallux, 1st/3rd/5th met heads †)	Loss of protective sensation
☐	Vibration	128 Hz fork at base of great toe; move proximally if absent	Dorsal column / large fibre — most sensitive
☐	Proprioception	Great toe, held by its sides; move proximally if absent	Sensory ataxia
☐	Reflexes	Plantar → ankle → knee	Reduced ankle jerks in neuropathy



10 g monofilament testing sites

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion: "I would perform urinalysis (ketones, leucocytes, nitrites ± albumin), and check HbA1c, UEC/eGFR, urine ACR and lipids †."
☐	Present by complication: glycaemic control clues → microvascular (eyes, nerves, kidneys) → macrovascular (carotids, heart, peripheral pulses) → feet

4. Quick reference

Complication	Examination findings
Retinopathy	Microaneurysms, dot-blot haemorrhages, hard exudates → cotton-wool spots → new vessels †
Neuropathy	Stocking sensory loss, ↓ vibration, absent ankle jerks, Charcot foot
Nephropathy	Oedema, ↑ BP; needs urine ACR †
Macrovascular	Carotid bruit, absent pulses, trophic changes, ↓ cap refill

#	Red flags (Deakin guideline)
1	Symptomatic hyperglycaemia ± dehydration
2	Ketosis >0.6 mmol/L or ketoacidosis
3	Severe infection / sepsis
4	Ulcer with depth to tendon or bone
5	Critical limb ischaemia
6	Pregnancy

Breast Examination — OSCE Checklist

Deakin MD2 · ELC7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Introduction → Inspection (seated) → Axillae + supraclavicular (seated) → Breast palpation (supine) → Close**. *Examine* **both breasts and draining nodes**. *Communicate throughout in your own words.*

1. Introduction (intimate examination)

☐	**Introduce** self (name, role); confirm patient ID
☐	**Explain** what the exam involves and why; check understanding
☐	**Offer a chaperone / observer** (document if declined)
☐	**Consent**: verbal, informed; "Just say stop and I will"
☐	**Wash hands**; gloves where indicated
☐	**Privacy**: undress behind curtain, drape/cover, warm room, good light
☐	Ask about symptoms, noticed changes, past surgery or radiotherapy
☐	Position: seated on the couch; drape so one breast/axilla is exposed while the other is covered

2. Inspection — seated, arms by sides

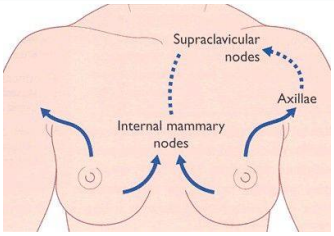
	Inspect for	What to say / significance
☐	Introduce step	"I'm going to start by looking at your breasts. Could you sit up straight and lower your gown when you're ready?"
☐	Size, shape, symmetry	Asymmetry may be normal — compare with patient's baseline
☐	Lumps or thickenings	Visible contour change
☐	Skin puckering / dimpling	Tethering by underlying tumour
☐	Peau d'orange	Pitted oedematous skin — lymphatic obstruction
☐	Erythema, prominent veins	Inflammation, infection, inflammatory carcinoma
☐	Sores or ulcers	On breast or nipple
☐	Nipple retraction	New drawing-in of the nipple
☐	Rash on / around nipple	Eczematous change — think Paget's disease of the nipple
☐	Axilla and upper arm	Axillary swelling or lumps; arm swelling (lymphoedema)
☐	Pendulous breasts*	Ask patient to lift breasts to inspect underneath

3. Inspection with movement

	Manoeuvre	Look for
☐	"Could you follow my arm movements?" — raise arms above head	Tethering: tissue, skin or nipple not moving smoothly or symmetrically
☐	Hands on hips, lean forward and press hands into hips (pectoral contraction)	Increased dimpling = fixation to chest wall

4. Palpation — lymph nodes (seated)

☐	Hold the patient's outstretched arm with yours to relax the axilla; palpate with your free hand (gloves may be preferred)
☐	Palpate all 5 walls: **anterior, posterior, medial, lateral, apex**
☐	Repeat on the other axilla — always examine both
☐	Palpate supraclavicular fossae both sides
☐	"Thank you. Pull the gown up, and when you're ready, please lie down."

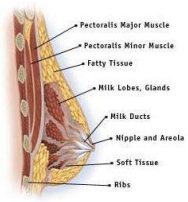


Lymph drainage of the breast

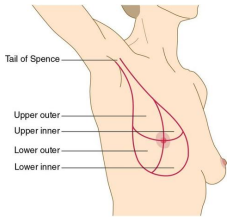
5. Palpation — breasts (supine)

☐	Uncover **one breast at a time**; patient's arm abducted with hand behind head
☐	Flat pulps of fingers, small circles: light pressure then deeper, before moving on (other hand may support tissue)
☐	Cover the whole area: clavicle → sternum → mid-axillary line → bra line
☐	Systematic pattern: vertical strips ("lawn mowing") or spiral out from the nipple; minimise lifting hand off skin
☐	Palpate behind the nipple for lumps
☐	Include the **axillary tail** (superolateral)
☐	Nipple discharge*: ask patient to express if discharge reported; note colour, single/multiple ducts †

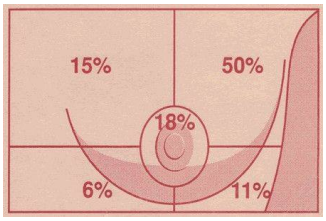
☐	Repeat fully on the other breast



Breast anatomy



Breast quadrants and axillary tail



Where breast cancers most commonly occur

6. If you find a lump — describe it

☐	Position	Quadrant, clock-face position and distance from areola
☐	Size	Diameter in cm (or pea / grape / plum)
☐	Shape and surface	Rounded, regular / irregular, smooth / rough
☐	Consistency	Fluctuant, soft, firm, hard
☐	Tenderness	How tender
☐	Mobility	Mobile or fixed — to skin or deep (chest wall)
☐	Borders	Discrete or poorly defined
☐	Retraction	Altered contour, dimpling
☐	Number	Single or multiple; check the other breast

7. Closing

☐	Thank patient; let them know they can dress; give privacy
☐	Explain findings and next steps; invite questions
☐	Document consent, chaperone, examination, findings and discussion
☐	Present: "On inspection the breasts were symmetrical with no skin or nipple changes. No masses on palpation of either breast or the axillary tails, and no axillary or supraclavicular lymphadenopathy. This is a normal breast examination."

8. Quick reference

Feature	Fibroadenoma †	Cyst †	Carcinoma †
Age	15–35	35–55, perimenopausal	Usually >40
Feel	Smooth, rubbery	Smooth, fluctuant / firm	Hard, irregular
Mobility	Very mobile ("breast mouse")	Mobile	Often fixed
Skin / nipple	Normal	Normal	Dimpling, peau d'orange, retraction

Key fact	From the Deakin guideline
Triple test	Clinical exam + imaging (mammogram/US; MRI gold standard) + non-excisional biopsy for **any** lump
BreastScreen	Free mammogram 2-yearly from 40; actively invited 50–74
Burden	Most common cancer in Australian females; 1 in 8 by age 85
Lymph drainage	Lateral/central → axillary nodes; medial → internal mammary (not palpable)
Commonest site	Upper outer quadrant †

- Breast examination alone is **not** recommended as a screening test (RACGP) — use it to teach breast awareness.
- Report promptly: new lump, nipple change or discharge, skin colour/texture change, pain, swelling.
- Pitfalls: exposing both breasts at once, missing the axillary tail or behind the nipple, forgetting supraclavicular nodes, no chaperone offer.

Inguinoscrotal Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Introduction → Standing: inspect, palpate, special tests → Supine: re-examine, reduce, transilluminate, percuss, auscultate**. *Not assessed practically in Year 2, but assessable in written form.*

1. Introduction (intimate examination)

☐	**Introduce** self (name, role); confirm patient ID
☐	**Explain** what the exam involves and why; check understanding
☐	**Offer a chaperone / observer** (document if declined)
☐	**Consent**: verbal, informed; "Just say stop and I will"
☐	**Wash hands**; gloves where indicated
☐	**Privacy**: undress behind curtain, drape/cover, warm room, good light
☐	Sheet provided; **gloves on both hands** for the whole examination
☐	Sign outside the cubicle so you are not interrupted
☐	Start with patient **standing**; undress lower garments behind curtain (gown if preferred)
☐	Ask patient to show you the area of concern; ensure adequate exposure (umbilicus to mid-thigh)

2. Inspection — standing

	Inspect	Look for
☐	Skin of lower abdomen, inguinal regions, upper inner thighs, scrotum	Scars, erythema, rashes
☐	Swellings / lumps	Groin or scrotal; note side
☐	Testicular asymmetry	Left usually hangs lower than right (normal)
☐	Scrotal oedema	Note extent
☐	Cough impulse — visible	"Please turn your head and cough": look for bulge each side. No impulse in obstructed, strangulated or omental hernia

3. Palpation — inguinal region (standing)

☐	Compare both sides: palpate from pubic tubercle to ASIS, above and below the inguinal ligament
☐	Inguinal lymph nodes; femoral artery pulse
☐	Any swelling: warmth, tenderness, consistency
☐	Extent: can you reach its top and bottom? Does it descend into the scrotum?
☐	Relation to **pubic tubercle** and inguinal ligament: above-medial = inguinal; below-lateral = femoral †

4. Palpation — scrotum (standing)

☐	Warn patient; ask them to say if there is any pain
☐	Palpate one testis at a time between thumb and fingertips (or cradle with middle + ring fingers, thumb above)
☐	Both testes present? Estimate size of each; compare
☐	Palpate testis, epididymis (posterior) and spermatic cord
☐	Warmth, tenderness, swellings or lumps; consistency
☐	Can you **get above** the swelling? Gently pinch the upper scrotum between thumb and fingers

5. Special tests (standing)

	Test	How	Interpretation
☐	Cough impulse — palpated	Fingers over pubic tubercle region; patient coughs; each side	Expansile impulse = hernia
☐	Deep ring occlusion	Reduce, then press over deep ring (midpoint of inguinal ligament †); cough; each side	Controlled = indirect; still protrudes = direct
☐	Cremasteric reflex*	Stroke upper inner thigh	Testis rises; lost in torsion

6. Supine

	Step	How	Interpretation
☐	Re-examine	Re-palpate any swelling found standing	Exact site and nature clearer supine
☐	Reducibility	Attempt gentle manual reduction; feel for gurgling	Irreducible = incarcerated; tender + irreducible = strangulation
☐	Transillumination	Darkened room, torch against scrotal swelling	Lights up = cystic; dark = solid
☐	Percussion	Percuss over swelling	Hollow = gas-filled bowel; splash = obstructed loop
☐	Auscultation	Over the mass and the abdomen	Bowel sounds = bowel in hernia

7. Closing

☐	Check for undue pain or distress
☐	Privacy to dress
☐	Explain findings and next steps; invite questions
☐	Restore examination area/couch
☐	Document consent, chaperone, examination, findings and discussion
☐	Completion statement: "I would also examine the abdomen and external genitalia †"

8. Quick reference

Hernia †	Site	Deep ring occlusion	Into scrotum?	Other
Indirect inguinal	Through deep ring, lateral to inferior epigastric vessels	Controlled	Often	Most common; younger men
Direct inguinal	Hesselbach's triangle, medial to inferior epigastric vessels	Not controlled	Rarely	Older; weak posterior wall
Femoral	Below and lateral to pubic tubercle	—	No	More common in women; high strangulation risk

Scrotal lump †	Get above?	Separate from testis?	Transilluminates?	Clues
Inguinoscrotal hernia	No	Yes	No	Cough impulse, may reduce
Hydrocele	Yes	No — testis within	Yes	Smooth, painless
Epididymal cyst	Yes	Yes (above / behind)	Yes	Small, smooth
Varicocele	Yes	Yes	No	"Bag of worms", left, empties lying down
Testicular tumour	Yes	No — part of testis	No	Hard, painless, irregular; urgent US
Torsion / epididymo-orchitis	Yes	—	No	Torsion: acute pain, high-riding, absent cremasteric — surgical emergency

Cervical Screening & Bimanual Pelvic Examination — OSCE Checklist

Deakin MD2 · ELC8 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Prepare and consent → Equipment → Position → Inspect → Speculum + CST → Bimanual → Close**. *Practised on part-task trainers — speak to the model as you would to a patient.*

1. Before the patient is on the couch

☐	Why a pelvic examination is needed; what it involves (speculum, CST, swabs, bimanual) and the position
☐	Past experience of pelvic examinations
☐	Obstetric history: children? vaginal births? (guides speculum size)
☐	Menstrual history: LMP; possibility of pregnancy
☐	Verbal consent; "Just say stop and I will"
☐	Offer a chaperone (advocate for patient and doctor; often a nurse who assists)
☐	Offer to empty bladder first
☐	Timing*: CST ideally ~2 weeks after day 1 of last period (if menstruating)

2. Equipment

☐	Gloves	Cushion / folded blanket to raise pelvis	Manoeuvrable light
☐	Lubricant	Range of speculum sizes	Swabs if needed
☐	Cytobrush + liquid-based vial (ThinPrep)	Single-use mat (bluey)	Sanitary pad and tissues
☐	Chaperone present	Vial labelled and form completed †	

3. Position and inspection

☐	Gather equipment; hand hygiene; gloves
☐	Patient undresses from waist down behind curtain and covers with blanket; enter with permission
☐	Position: supine, knees drawn up, heels together, let knees fall apart (do **not** push knees apart)
☐	Expose waist down only when ready; adjust light
☐	Inspect external genitalia: vulval skin, discharge, lesions, prolapse †



Position: heels together, knees fall apart

4. Speculum and cervical screening test

☐	Warn the patient; part labia with non-dominant hand
☐	Insert lubricated speculum with blades sideways, rotating during insertion as patient breathes out; aim towards sacrum, not head
☐	Gently open blades until cervix is visualised
☐	Fix speculum open; inspect cervix: os, ectropion, polyps, discharge, lesions †
☐	Insert cytobrush into the os and rotate 2–3 times
☐	Remove brush and agitate well in the ThinPrep liquid
☐	Loosen speculum and remove gently as patient breathes out (watch vaginal walls †)

5. Bimanual examination (if indicated)

☐	Warn patient; gently touch the introitus, then slowly insert 2 lubricated gloved fingers
☐	Palpate cervix: position, consistency, os, tenderness †
☐	**Cervical excitation**: gently move the cervix side to side — watch the face for pain
☐	Uterus: other hand on lower abdomen pressing down towards internal fingers — size, shape, position (anteverted / retroverted), mobility, tenderness †
☐	Adnexa: internal fingers in each lateral fornix, abdominal hand over iliac fossa — masses, tenderness; both sides
☐	Withdraw fingers; check glove for blood or discharge

6. Closing

☐	Cover patient; tell them the examination is complete; leave tissues and bin; privacy to dress
☐	Explain findings and how/when CST results will be received; follow-up plan
☐	Invite questions; check comfort
☐	Document consent, chaperone, findings, tests taken, follow-up

7. Quick reference

Bimanual finding †	Think of
Cervical excitation (motion tenderness)	Pelvic inflammatory disease, ectopic pregnancy
Enlarged, irregular, firm uterus	Fibroids; pregnancy (smooth, soft)
Bulky, tender uterus	Adenomyosis
Fixed, retroverted, tender uterus ± nodules	Endometriosis
Adnexal mass	Ovarian cyst or tumour, ectopic, tubo-ovarian abscess

- Bimanual examination is **not** recommended routinely in asymptomatic people attending for screening; it's for symptoms and pregnancy.
- Australian NCSP: HPV-based CST every 5 years for people with a cervix aged 25–74; self-collection is an option for everyone †.
- Clinician collection (HPV + liquid-based cytology) is required for symptoms (PCB, IMB, PMB, persistent unexplained discharge) and for AIS surveillance.
- Pitfalls: blades inserted vertically or aimed upwards, closing blades on the cervix, no chaperone offer, not warning before touching, not watching the face.

HPV Self-Collection Discussion — OSCE Checklist

Deakin MD2 · ELC8 · Counselling station

Tick each point as you cover it. **Shaded rows (*)** *are covered if relevant.* † *= added from standard teaching, not in the Deakin guideline. Use plain language and check understanding.*

1. Opening

☐	Introduce self, confirm patient ID, explain purpose of the discussion
☐	Ask what the patient knows about cervical screening; screening history (last test, results)
☐	Explore concerns (past experiences, cultural or personal preferences)

2. Check eligibility

	Check	Ask about	If yes
☐	Offer to all	Self-collection is an option for anyone in routine cervical screening	Highly acceptable to under- and never-screened people
☐	Symptoms?	Post-coital, intermenstrual or post-menopausal bleeding; persistent unexplained discharge	Contraindicated → clinician-collected HPV + cytology
☐	Surveillance?	Follow-up after completely excised adenocarcinoma in situ (AIS)	Contraindicated → clinician collection

3. Explain why it's worthwhile

☐	As sensitive as a clinician-collected test; easy to do
☐	Over 70% of Australians diagnosed with cervical cancer were under-screened or never screened
☐	Priority groups*: CALD communities, people with disability, LGBTQIA+ people, rural and remote, Aboriginal and Torres Strait Islander people

4. Teach the technique

☐	Wash hands; find a private, comfortable position †
☐	Twist the cap and remove the swab
☐	Insert swab into the vagina about the length of an index finger
☐	Rotate for at least **10 seconds**
☐	Place swab back in the tube and screw the cap on
☐	Do **not** put the swab down on any surface between opening and closing
☐	Return the sample to the healthcare provider

5. Results and close

☐	Sample goes to a pathology lab for HPV testing; results go to the healthcare provider
☐	Explain follow-up: HPV not detected → rescreen in 5 years; HPV detected → may need a clinician-collected sample or colposcopy †
☐	Teach-back: ask the patient to explain the steps; invite questions; thank patient
☐	Document discussion and plan

Shoulder Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *Examine the normal side first if one side is painful †.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: usually start standing
☐	Exposure: sleeveless top at least (singlet ideal); compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both sides, from front, side and back			
☐	Posture	Shoulder height, position of arm	Guarding, dislocation
☐	Skin and scars	Erythema, bruising, arthroscopy or surgical scars	Infection, previous surgery
☐	Swelling	Over joint, AC joint	Effusion, AC injury
☐	Wasting	Deltoid, supraspinatus and infraspinatus fossae	Rotator cuff tear, nerve injury
☐	Deformity	Step at AC joint, squared-off shoulder, winged scapula †	AC dislocation, glenohumeral dislocation
Feel — start medially and work round to the back			
☐	Temperature	Back of hand over both shoulders	Warm = inflammation, infection
☐	SC joint → clavicle → AC joint	Palpate along the bones	Tenderness, step, swelling
☐	Head of humerus	Anterior and lateral	Tenderness
☐	Long head of biceps	Bicipital groove (arm externally rotated †)	Biceps tendinopathy
☐	Scapula	Borders and spine	Tenderness
☐	Crepitus	Hand over joint while moving it through a limited range	OA, rotator cuff disease
Move — active, then passive if limited (watch from front and back)			
☐	Abduction	"Lift your arms out to the side and up" (0–180°); palpate for crepitus	Painful arc 60–120° = impingement †
☐	Adduction	"Bring your arm across your body"	
☐	Flexion	"Lift your arms forwards and up" (0–180°)	
☐	Extension	"Move your arms backwards" (0–60°)	
☐	Internal rotation	Hand behind back, reach up the spine (note highest level †)	Frozen shoulder, cuff
☐	External rotation	Elbows tucked in at 90°, rotate forearms out (0–60°)	Lost first in frozen shoulder †
☐	Measure	Limited → estimate/measure degrees; otherwise "full range of motion"	
Special tests			
☐	Empty can (Jobe's)	Arm at 90° elevation, 30° forward (scapular plane), thumbs down; push down while patient resists	Pain / weakness = supraspinatus (rotator cuff)
☐	Hawkins (impingement)	Shoulder and elbow flexed to 90°; your arm as fulcrum, internally rotate the shoulder	Pain = subacromial impingement; negative = unlikely
☐	Apprehension (supine)	Shoulder 90° abducted, elbow 90° flexed; gently externally rotate	Pain or apprehension = anterior instability
☐	Clunk test	Supine, arm fully abducted; full external rotation, push humeral head slightly anteriorly	Grinding / clunk = labral tear †
☐	Sulcus sign	Arm at side, neutral rotation; pull down at the elbow	Gap >1 fingerbreadth below acromion = inferior instability



Empty can test



Figure 2. The Hawkins test

Hawkins test



Apprehension test (supine)

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the neck and elbow (joints above and below) and do a neurovascular exam of the upper limb †."
☐	Present: "On inspection there was no wasting, swelling or deformity. No tenderness or warmth. Full active and passive range of motion without pain. Special tests for rotator cuff, impingement and instability were negative."

4. Quick reference †

Pattern	Key findings
Rotator cuff tear / tendinopathy	Weak, painful abduction; positive empty can; wasting of supra/infraspinatus
Subacromial impingement	Painful arc 60–120°; positive Hawkins
Frozen shoulder (adhesive capsulitis)	Global loss of active AND passive movement, external rotation first
Anterior instability	Previous dislocation; positive apprehension
AC joint pathology	Tenderness/step at AC joint; painful arc at the top (170–180°)

Elbow Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: usually start standing
☐	Exposure: sleeveless top at least; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both sides, from front, side and back			
☐	Carrying angle	Arms by sides, palms forward	Cubitus valgus / varus (old fracture)
☐	Swelling	Over the joint and olecranon	Effusion, olecranon bursitis
☐	Skin	Erythema, scars, psoriatic plaques †	
☐	Nodules	Rheumatoid nodules, gouty tophi (extensor surface)	RA, gout
☐	Wasting	Forearm muscles	
Feel			
☐	Temperature	Back of hand, compare sides	Warm = inflammation, infection
☐	Lateral epicondyle	Palpate	Tender = tennis elbow
☐	Medial epicondyle	Palpate	Tender = golfer's elbow
☐	Olecranon tip	Palpate	Bursitis, fracture
☐	Discrete swellings	Nodules, bursa	
☐	Effusion	Thumb along edge of ulna just distal to olecranon; fully extend the elbow	Palpable bulge = fluid
Move — active, then passive if limited			
☐	Flexion	"Bend your elbow" (0–150°)	
☐	Extension	"Straighten your elbow" (0°)	Loss of extension = effusion, OA
☐	Pronation	Elbows tucked in at 90°: "Turn your palms down" (0–80°)	
☐	Supination	"Turn your palms up" (0–80°)	
☐	Crepitus	Feel during passive movement	OA
☐	Measure	Limited → degrees; otherwise "full range of motion"	
☐	Ulnar nerve*	If deformity or numbness: neuro exam of the hand and arm	Ulnar nerve entrapment
Special tests			
☐	Lateral epicondylitis	Elbow 90°, forearm pronated, wrist flexed; support elbow; patient extends wrist against resistance	Pain at lateral epicondyle = tennis elbow
☐	Medial epicondylitis	Elbow 90°, forearm supinated; support elbow; patient flexes wrist against resistance	Pain at medial epicondyle = golfer's elbow

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the shoulder and wrist (joints above and below) and do a neurovascular exam of the upper limb †."
☐	Present: "Normal carrying angle with no swelling, nodules or scars. No tenderness over the epicondyles or olecranon and no effusion. Full range of flexion, extension, pronation and supination. Tests for lateral and medial epicondylitis were negative."

Hand & Wrist Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *Includes* **Measure** *(function) for the hand.*

1. Introduction

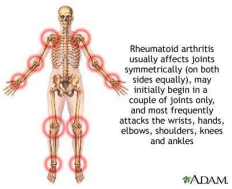
☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: seated on bed (or at a table), hands resting **palms down on a pillow**
☐	Exposure: hands and arms to the elbows
☐	Ask about pain before touching
☐	General observation: systemically well? in pain? age; other joint or skin disease (psoriasis); walking aids, splints

2. Examination

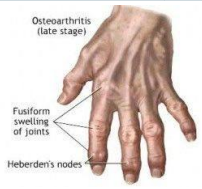
	Step	What to do / say	Abnormal
Look — forearms, wrists and hands (dorsal, then palms †)			
☐	Distribution	Symmetry, which joints are affected	Symmetrical small joints = RA; DIPs = OA, psoriatic
☐	Skin	Erythema, atrophy, rashes, scars (carpal tunnel or tendon release)	Psoriasis, previous surgery
☐	Nails	Pitting, onycholysis	Psoriatic arthritis
☐	Swelling	Joint swelling; "sausage" fingers	Synovitis; dactylitis (psoriatic)
☐	Deformity	Ulnar deviation, swan neck, boutonnière, Z-thumb; Heberden's/Bouchard's nodes †	RA; OA
☐	Wasting	Thenar or hypothenar eminence, dorsal interossei	Median / ulnar nerve lesion
☐	Function	Watch how they use their hands informally	



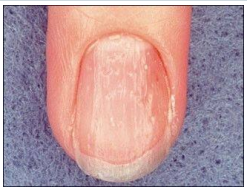
Rheumatoid arthritis — late changes



RA joint distribution



Osteoarthritis — Heberden's nodes



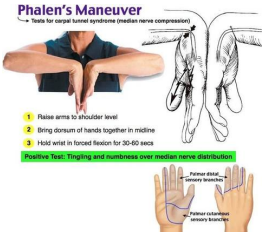
Nail pitting (psoriasis)



Psoriatic arthritis

Feel			
☐	Temperature	Back of hand over wrists and joints	Warm = active inflammation
☐	Wrists	Palpate with both thumbs	Tenderness, synovitis
☐	MCP joints	Squeeze across MCPs gently, then palpate each †	Tenderness = synovitis (early RA)
☐	PIP and DIP joints	Palpate each joint	Boggy (synovitis) vs hard bony (OA)
☐	Joint features	Tenderness, synovitis/effusion, warmth, subluxation	
☐	Scaphoid*	After trauma: anatomical snuffbox and scaphoid tubercle (thenar)	Scaphoid fracture
☐	Pulses	Radial and ulnar pulses; capillary refill	Vascular compromise
Move — active			
☐	Wrist	Prayer and reverse prayer: flexion and extension †	Reduced range
☐	Fist and straighten	"Make a fist, then straighten your fingers"	Screen MCP/IP movement
☐	Thumb opposition	Touch thumb to each fingertip	
Move — passive			
☐	Wrists, MCP, PIP, DIP	Move each through range; feel for crepitus	Restriction, crepitus
Measure — hand function			
☐	Grip strength	"Squeeze my fingers"	Weak grip
☐	Key grip	Use lock and key to open lock	
☐	Opposition strength	Thumb to each finger, try to pull apart	Median nerve weakness

☐	Practical tasks	Undo screw-top jar; do up and undo a button; write with a pen	Functional impact
Special tests			
☐	Phalen's	Hold wrists fully flexed (backs of hands together) for 60 s †	Tingling in median distribution = carpal tunnel
☐	Tinel's	Tap over the flexor retinaculum (carpal tunnel)	Reproduces symptoms (less reliable)
☐	Neuro screen*	Sensory islands and key muscles of median, ulnar and radial nerves (covered in B&B)	Nerve injury — document sensation, movement, circulation after trauma



Phalen's manoeuvre

3. Closing

☐	Check the elbows: rheumatoid nodules, gouty tophi, psoriatic plaques
☐	Thank patient; wash hands
☐	Present: "There is no deformity, swelling or wasting, and the skin and nails are normal. No tenderness or synovitis. Full active and passive range of movement; grip, key grip and opposition are normal and function is preserved. Phalen's and Tinel's were negative."

4. Quick reference †

Feature	Rheumatoid arthritis	Osteoarthritis	Psoriatic arthritis
Joints	Symmetrical MCPs, PIPs, wrists; DIPs spared	DIPs, PIPs, thumb base (1st CMC)	DIPs, asymmetrical; dactylitis
Swelling	Soft, boggy synovitis	Hard, bony	Sausage digits
Deformities	Ulnar deviation, swan neck, boutonnière, Z-thumb, MCP subluxation	Heberden's (DIP) and Bouchard's (PIP) nodes, squaring of thumb base	Telescoping (arthritis mutilans)
Other	Rheumatoid nodules, wasting of small muscles	Crepitus	Nail pitting, onycholysis, plaques

Hip Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8 min (+2 min reading)

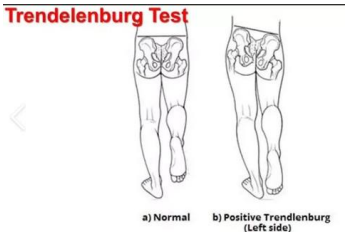
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1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: underwear ideally (shorts for comfort); shirt lifted to waist; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

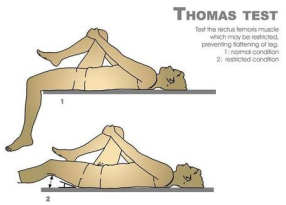
2. Examination

	Step	What to do / say	Abnormal
Look — standing			
☐	Walking in	Walking stick or other aid	
☐	Gait	Normal? Antalgic (short stance on painful side)? Trendelenburg (waddling) †	Hip pain, abductor weakness
☐	Inspect front, back, sides	Deformity, scars/wounds, gluteal and quadriceps wasting, symmetry	Previous surgery, disuse
☐	Trendelenburg test	Stand on one leg (hold your hands †); watch iliac crests; repeat on the other leg	Pelvis drops on the lifted side = weak abductors on the **standing** side



Trendelenburg test

Look — supine			
☐	Re-inspect	Anything missed; leg rotation or shortening †	Fractured NOF: shortened, externally rotated
Feel			
☐	Hip joint	Just distal to midpoint of inguinal ligament; watch the face	Joint tenderness
☐	Greater trochanters	Palpate both	Trochanteric bursitis
Move — supine (active, then passive if pain or restriction)			
☐	Flexion	Straight leg raise (to 90°); knee bent (to 120°)	
☐	Thomas test	Hand under lumbar spine; flex one hip fully until back is flat; watch the other leg	Other leg lifts = fixed flexion deformity
☐	Internal / external rotation	Hip and knee at 90°, rotate foot out (internal) and in (external) †	Internal rotation lost first in OA †
☐	Abduction / adduction	Stabilise the pelvis with your other hand on the opposite ASIS †	
☐	Extension	Patient prone: lift leg off bed	



Thomas test

Measure			
☐	True leg length	ASIS → medial malleolus, both sides	Difference = bone shortening (fracture, congenital, surgery)
☐	Apparent leg length	Umbilicus → medial malleolus, both sides	Difference = pelvic tilt, adduction/abduction, spinal curvature
Special tests (confirm done)			
☐	Trendelenburg, Thomas, leg lengths	Usually completed in Look / Move / Measure	

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the lumbar spine and knee (joints above and below) and do a neurovascular exam of the lower limb †."
☐	Present: "Normal gait with no aids. No deformity, wasting or scars; Trendelenburg negative. No tenderness. Full range of movement; Thomas test negative; true and apparent leg lengths equal."

Knee Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8 min (+2 min reading)

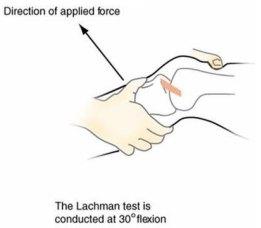
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1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: underwear ideally (shorts for comfort); see the whole limb and quadriceps; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — standing			
☐	Inspect front, back, sides	Deformity, scars/wounds, quadriceps wasting, swelling/effusion, symmetry	Effusion, previous surgery
☐	Alignment	Valgus / varus; hyperextension	OA (varus common †)
☐	Popliteal fossa	Baker's cyst	OA, RA
☐	Gait	Normal? Antalgic?	
Look — supine			
☐	Re-inspect	Anything missed	
Feel			
☐	Temperature	Compare both knees	Warm = inflammation, infection
☐	Palpate	Patella, joint lines (knee flexed †), tibial tuberosity, collateral ligaments; watch the face	Joint-line tenderness = meniscus
☐	Bulge / sweep test	Knee extended; hand on suprapatellar pouch; sweep medial side up, then lateral side down — watch medial side †	Bulge = small effusion (preferred test)
☐	Patellar tap	Knee extended; hand above patella pushing fluid down; two fingers press patella	Tap = large effusion
Move			
☐	Active flexion / extension	"Bend your knee… straighten it" (0–140° †)	Limited → measure degrees
☐	Passive	Feel for crepitus through flexion and extension	OA, patellofemoral
Special tests — cruciates			
☐	Posterior sag	Both knees flexed to 90°, look from the side	Tibia sags back = PCL tear
☐	Anterior drawer	Knee 90°, sit gently on foot, grasp tibia, pull forward	>6 mm translation = ACL tear
☐	Posterior drawer	As above, push tibia back	Excess backward movement = PCL tear
☐	Lachman's	Knee 20–30°; hold distal thigh, pull proximal tibia forward	Translation / soft end point = ACL



Lachman's test (20–30° flexion)

Special tests — collaterals (knee at 30°)			
☐	MCL	Valgus stress (push knee inwards)	Medial gaping = MCL tear/laxity
☐	LCL	Varus stress (push knee outwards)	Lateral gaping = LCL tear/laxity
Special tests — menisci			
☐	McMurray's — medial	Full flexion; external rotation of tibia + valgus stress, then extend; palpate joint line	Pain, click, locking = medial meniscus
☐	McMurray's — lateral	Full flexion; internal rotation + varus stress, then extend	Pain, click, locking = lateral meniscus
☐	Apley's grind	Prone, knee 90°, stabilise thigh; press down on foot and rotate	Pain or clicking = meniscus
Special tests — patella			
☐	Patellar apprehension	Knee extended; push patella laterally while flexing the knee	Apprehension = patellar instability

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the hip and ankle (joints above and below) and do a neurovascular exam of the lower limb †."
☐	Present: "Normal gait; no deformity, wasting or effusion. No warmth or tenderness. Full range of movement without crepitus. Cruciate, collateral, meniscal and patellar tests were all negative."

Ankle & Foot Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: legs exposed to the knees; shoes and socks off; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both limbs, front, side and back			
☐	Gait	Normal? Antalgic?	
☐	Swelling, scars, deformity	Ankle and foot	
☐	Wasting	Calf, small foot muscles	
☐	Nails	Psoriatic changes	Psoriatic arthritis
☐	Arches	Transverse (under MTPs) and longitudinal (1st MTP to heel)	Flattened in RA (pes planus)
☐	Calluses	Soles	Abnormal pressure
☐	Forefoot	Hallux valgus, clawing, crowding of toes, sausage toes	Bunion; RA; dactylitis
☐	Footwear*	Wear pattern on shoes †	
Feel			
☐	Temperature	Compare both sides	Warm = inflammation, gout, infection
☐	Ankle joint line	Swelling, tenderness	
☐	Malleoli and ligaments	Both malleoli; medial and lateral ligaments	Fracture, sprain
☐	Base of 5th metatarsal	Palpate	Avulsion fracture
☐	Achilles tendon	Nodules, tenderness, gap †	Tendinitis, rupture, RA nodules
☐	MTP squeeze	Compress 1st and 5th metatarsals together between thumb and finger	Tender = MTP synovitis (early RA)
☐	MTP and IP joints	Palpate individually	Tenderness, swelling; 1st MTP = gout †
Move			
☐	Ankle	Dorsiflexion and plantarflexion (active, then passive)	
☐	Subtalar joint	Inversion and eversion (hold the heel †)	
☐	MTP and IP joints	Passive flexion and extension of each	Hallux rigidus †
Special tests			
☐	Knee-to-wall	Heel flat, bend knee to touch wall; move foot back until heel lifts or pain; measure great toe to wall	Normal ≥ 9 cm; less = reduced dorsiflexion
☐	Thompson / Simmonds	Patient kneels on a chair; palpate Achilles; squeeze calf just below its widest point	No plantarflexion = Achilles rupture
☐	Mulder's sign	Squeeze the metatarsals together and press up from the sole between 3rd and 4th toes, just proximal to MTPs	Pain (± click) = Morton's neuroma

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the knee, do a neurovascular exam of the foot, and assess footwear †."
☐	Present: "Normal gait. No swelling, deformity or skin changes; arches preserved. No tenderness, including over the malleoli and base of the 5th metatarsal. Full range at the ankle, subtalar and toe joints. Knee-to-wall over 9 cm; Thompson and Mulder's tests negative."

4. Quick reference †

- **Ottawa ankle rules**: X-ray if malleolar pain plus bone tenderness at the posterior edge or tip of either malleolus, or unable to weight-bear 4 steps. Foot X-ray if tender at the base of the 5th metatarsal or navicular.
- Acute hot, swollen 1st MTP = think gout; exclude septic arthritis if febrile or unwell.